Find the problem.
Then earn the evaluation.

QR exercise | Prompt #1: finding potential customers before repeatable sales are proven

Optional second round (about 15 minutes)

The official State Audit Office page/PDF did not load; its findings were read through BMG, not verified against the original. Open the official link in Sources, search for EMR/EHR compatibility and reporting gaps, and paste the relevant paragraphs with: Run Steps 1 to 4 again with this.

Summary

Problem: patient information and hospital work do not always connect, making coordination and reliable reporting harder.
Who: Georgian private multi-specialty hospitals and groups; owner/CEO buys, clinical/operations staff champion, finance and IT evaluate.
Search: Georgian and English; Georgia the country, not the US state.
Pain words: incomplete records, handoffs, repeated entry, reporting, documentation and reimbursement corrections.
Workarounds: spreadsheets, calls and manual checks are hypotheses unless a source identifies them.
Alternatives: existing hospital systems, add-on reporting and manual coordination; no unverified competitor install claims.
Exclude: patients' private experiences, non-hospital businesses, institutions outside supportable scope and tariff problems software cannot solve.
Scope: Phase 1 connected base system first; workflow AI and the proactive brain are later, separately agreed phases.

What this research actually found

Seven relevant source records, not 20 qualifying posts. Two historical executive interviews; four related C/Can project pages; one audit news report. Five named research contacts, but **zero fully verified current purchase opportunities**. Project participation is not an individual's statement of pain, and related project pages are not independent demand signals.

Company context used

MediMind, medimind.md. Your 8 October explainer and 22 September buyer-pain research were used, with your later Phase 1 clarification taking precedence over older AI-inclusive copy. Your anchor agreement covers GeoHospitals' 25-hospital network with staged implementation; it is not proof of 25 independent wins or measured outcome improvement. This exercise prepares discovery only. No messages were sent.

Founder offer: for the next 10 hospital clients, an included parallel base-system evaluation before purchase commitment; current software stays in place. Price matching and continuing base-product development have agreed scope and written terms. About two weeks is a target after readiness, not an unconditional launch guarantee.

Search the buyer's words

Summary continued | Twelve focused queries used in this research

English	Why search it
Georgia hospital CEO digital transformation fragmented systems patient data	Look for leaders naming information gaps; reject US-state results.
Georgia hospital clinic software testimonial	Find attributed experiences, not vendor promises.
Georgia hospital manual processes -Georgian Bay -site:scribd.com	Look for specific recurring work.
hospital Tbilisi fragmented	Find coordination problems, not merely a fragmented market.
Vian hospital digital Georgia	Check what an existing group says, not what we assume.
site:citycancerchallenge.org Tbilisi information systems	Find local projects, named participants and documented gaps.

Georgian	Meaning / purpose
საქართველო კლინიკა დირექტორი ციფრული სისტემა პაციენტის ინფორმაცია	Clinic director, digital systems and patient information.
ჰოსპიტალი ელექტრონული სამედიცინო ჩანაწერები გამოწვევები დირექტორი	Hospital electronic records, challenges and directors.
კლინიკები ბიუროკრატია დოკუმენტაცია	Documentation and administrative work.
კლინიკა დოკუმენტაციის ხარვეზები დირექტორი	Directors describing documentation gaps.
ჰოსპიტალსი DRG 2026	Recent reimbursement discussion; distinguish policy from workflow.
საავადმყოფო ელექტრონული პრობლემები	Electronic-system problems in hospital work.

Recency rule

The preferred 90-day window is **14 July to 12 October 2026**. An undated live page is not a recent post. Publication dates in articles were checked separately from site headers and search-engine crawl dates. Neither executive pain interview below falls inside that window.

Method: open original pages when available; paraphrase, do not invent quotes or roles; inspect official hospital sites for fit. Exclude personal patient reviews, international look-alikes and generic vendor claims from lead evidence. A linked employer or project is not a warm introduction.

What the sources show

Watch | Seven opened records, with the limits kept visible

Record / date	One pain / workaround	What it establishes
S1 | C/Can: quality cancer management Undated; checked 12 Oct	Inconsistent coordination between cancer services. Project response: multidisciplinary teams and monitoring.	Local sector problem and named participants, not each participant's own complaint or a software buying request.
S2 | C/Can: Cancer Registry II Undated; checked 12 Oct	Incomplete and inconsistent registry data. Response: data-quality work, infrastructure assessment and training.	Reporting/data-quality need; NCDC is a public-system stakeholder, not our private-hospital buyer.
S3 | C/Can: registry data collection Undated; checked 12 Oct	Non-uniform data collection and consent. Response: harmonised collection and authorisation.	Related programme evidence; do not count it as independent confirmation of S2.
S4 | C/Can: multidisciplinary cancer care Undated; checked 12 Oct	Disconnected diagnostic, treatment and care specialists. Response: MDTs and common protocols.	Coordination need; software alone does not establish effective teamwork or better outcomes.
S5 | Koka Fruidze, Consilium Medulla director, BMG 14 Jan 2026	DRG reimbursement restrictions for complex cases. No hospital workflow workaround identified.	Firsthand financial concern, but principally policy/tariff pain. Not evidence of bad hospital software.
S6 | Archil Chukhrukidze, Tbilisi Heart Centre director at publication, BMG 14 Jun 2023	Ambiguous documentation-related enforcement and inability to challenge decisions effectively. No workaround identified.	Historical executive concern; not proof of poor records or present software-replacement intent.
S7 | BMG reporting State Audit Office findings 14 Feb 2025	Gaps in electronic reporting and EMR/EHR compatibility testing.	Secondary sector context only. Hospital in the example is unnamed; cannot identify a buyer from it.

Short source wording: S1: 'inconsistent coordination across services and facilities'; S2: 'suboptimal data completeness'; S4: 'No integration exists between diagnostics, treatment and care specialists'. Dates were not present on those project pages. These describe their project assessments, not current conditions at every participating hospital.

Excluded rather than padded

Doctra's live page displayed unrelated casino content and did not substantiate the cached testimonials: excluded. A jobs.ge IT-support record became empty on re-check: excluded. Generic product pages and hospital privacy notices establish neither acknowledged pain nor switching intent. No list was padded to reach 20.

Rank pain, not logos

Rank and Verify | Clarity is not willingness to buy

Rank / source	Single pain	Clarity / freshness	Why
1 | S4	Cross-specialist coordination	3 / 1*	Specific and relevant to connected workflows.
2 | S2	Incomplete registry information	3 / 1*	Clear data-quality issue; public-system owner.
3 | S3	Inconsistent collection/consent	3 / 1*	Specific, but overlaps S2.
4 | S1	Service coordination	3 / 1*	Relevant context and contact map; overlaps S4.
5 | S5	Complex-case reimbursement restrictions	3 / 1	Named executive, but software cannot set payer rates.
6 | S6	Documentation enforcement uncertainty	2 / 1	Named executive; old and legally focused.
7 | S7	Missing electronic reporting	2 / 1	Audit context; no identifiable hospital buyer.

Clarity: 3 specific, 2 mixed, 1 vague. Freshness: 3 within 30 days, 2 within 60, 1 older. ***Undated records receive a conservative 1, with recency UNSURE; it does not mean their age is known.** Same-score ties prioritise the closest Phase 1 problem. This ranks records, not people or purchase readiness.

Evidence group	Real customer/problem match	Current	Aware	Status
S1-S4: C/Can	UNSURE: organisational problem, no individual buyer testimony	UNSURE: undated	PASS for programme; UNSURE for each contact	Research only
S5: Consilium interview	FAIL as Phase 1 software pain: tariff/policy complaint	UNSURE: outside 90 days	PASS for stated policy pain	Remove from qualified leads
S6: Heart Centre interview	UNSURE: documentation concern, not data defect	UNSURE: 2023	PASS for historical concern	Research only
S7: audit reporting	FAIL as identifiable customer: hospital unnamed	UNSURE: 2025	UNSURE: no identified buyer	Context only

Strict shortlist result

No prospect passes all three tests. The five contacts on the next page form a separate discovery queue, not the prompt's verified top five. Do not call them qualified leads, high-intent prospects, confirmed champions or warm connections.

Five people worth learning from

Shortlist | Provisional research queue, not verified purchase opportunities

Account / contact	Public basis	What to confirm next
Todua Clinic Maka Maglakelidze C/Can lists: Manager, Todua Dighomi [S1]	Multi-profile institution confirmed [S9]. Official site also advertises a patient app with AI-assisted booking. No need to sell generic AI awareness.	Is this person the right current workflow owner? Where do handoffs still require extra work? Official leadership: Anri Chkhaidze, Chief Executive Director [S13].
American Hospital Tbilisi Natia Jokhadze Clinical oncologist listed by C/Can [S1]	Official hospital site opened [S10]; linked to local care-coordination work. Its existing systems are not evidence of dissatisfaction.	Confirm current role and a cross-team workflow. Then identify operations, finance and IT co-reviewers and the economic buyer.
Caucasus Medical Centre Ivane Kiladze Clinical oncologist listed by C/Can [S1]	Multi-specialty hospital and part of Vian [S11]. This is a group-purchasing question, not automatically one new client per hospital.	Confirm sponsorship and whether Vian centrally controls the software decision. Do not count multiple group sites as independent wins.
Mardaleishvili Medical Centre Nia Sharikadze Oncology department head listed by C/Can [S1]	Specialised oncology hospital with diagnostics, surgery and intensive care [S8]. **Adjacent-segment hypothesis**, not a proven fit for our original multi-specialty focus.	Verify scope/support fit first. Economic-buyer route: Giorgi Loladze, Executive Director, on official management page [S12].
Tbilisi Heart Centre Archil Chukhrukidze Director at 2023 publication [S6]	Historical documentation-related concern. Current role, ownership, scale and software needs remain unverified.	Reserve only: confirm identity/role and whether a present documentation problem exists. Drop if only a policy dispute or outside our supportable scope.

How to use the network

Hypothesis: ask your existing clinical and 500 network whether anyone genuinely knows these professionals. C/Can is a public relationship map, not evidence you have a connection. Never claim a referral that did not happen. A clinician can help surface a problem; purchasing still needs an accountable hospital sponsor.

No direct professional invitation or verified personal contact channel was found for these five. No email addresses were guessed. Roles from undated C/Can pages need confirmation. The queue is ordered for practical discovery, not by independently measured pain severity.

Ask first. Do not pitch yet.

Reach | Conditional drafts, each under 60 words; nothing sent

Static project pages are not normal reply threads. Use these only after a genuine introduction or a verified invitation to professional contact. They are not instructions to post a comment or send an unsolicited personal message.

Todua | Maka Maglakelidze

C/Can lists you on its Tbilisi quality cancer management team. When a patient's diagnosis, treatment plan and follow-up involve different departments, which part of keeping everyone working from the same information takes the most effort today?

American Hospital Tbilisi | Natia Jokhadze

I saw your name on C/Can's Tbilisi quality cancer management project. How does your team prepare the full patient picture for a treatment discussion when information comes from several departments?

Caucasus Medical Centre | Ivane Kiladze

C/Can lists you in its Tbilisi care-quality project. What have you tried to make patient-information handoffs between diagnostic and treatment teams easier, and what still needs manual coordination?

Mardaleishvili | Nia Sharikadze

I came across the C/Can quality cancer management project you lead. Which coordination problem remains hardest to solve inside a hospital, rather than through changes to the wider health system?

Heart Centre | Archil Chukhrukidze, only after role confirmation

In your 2023 BMG interview, you discussed uncertainty around documentation-related inspections. Is assembling reliable documentation still a practical challenge for your team, and how do you handle it today?

What to listen for

One recent example. How often it repeats. Who does the extra work. What it costs in time or money. What they have tried. Why changing the current system is difficult. Ask for process examples, not identifiable patient records.

Success

Across the **seven records**, coordination appears in two related C/Can pages; registry data quality appears in two related pages; policy/reimbursement, enforcement/documentation and electronic reporting each appear in one record. These are overlapping institutional sources, not seven independent buyer interviews. Financial loss, repeat entry and spreadsheet work remain hospital-specific hypotheses, not quantified findings.

Turn learning into a real test

What to change and Final check | A proposed next step, not a forecast

Targeting hypothesis

Prioritise hospital workflow owners connected to real improvement programmes, then confirm a recurring internal problem. Multi-specialty hospitals remain the main market; specialised oncology is a separate hypothesis. Keep public-system contacts as research/connector possibilities, not paying-hospital leads.

Positioning hypothesis

Lead with keeping patient information and cross-team work connected. Test whether the buyer cares most about a slower handoff, hard-to-trust reporting or costly documentation corrections. Do not assume every reimbursement shortfall is caused by software. Avoid claiming full replacement is the only way any AI can work.

Offer hypothesis

Begin with one workflow walkthrough and a baseline. If there is enough fit, agree a limited Phase 1 parallel evaluation within the next-ten offer: named hospital reviewers, scope, data permission, success criteria and a decision date. This tests the broader replacement; it is not an autonomous-AI pilot.

Suggested journey

Fit-check hospital -> genuine route in -> confirm recurring pain -> identify sponsor and champion -> agree scope, hospital approvals and permitted data -> launch parallel base system -> staff test it -> compare results -> purchase decision -> staged rollout.

Data permission and delivery are separate milestones. A launched environment is not a completed hands-on evaluation, a qualified lead or a sale.

Proposed first cycle: five genuine introductions/permission checks, three completed workflow conversations, one evaluation plan if fit is confirmed. These are proposed activity targets, not predicted conversion rates. Your working outcome metric remains new hospital buying organisations beyond the anchor partner completing their first hands-on parallel-system evaluation in the last 30 days.

Final check

Verdict: there is public evidence of local coordination and data-quality gaps, but not yet a verified set of current hospital buyers asking for our replacement.
Assumptions: Georgian private hospitals remain the first market; repeatable sales are not established by the anchor agreement; programme participants may be useful research contacts; oncology may be a supportable adjacent segment; a genuine introduction may exist in your network. None is proof of demand.
UNSURE: project dates/currentness; each contact's present role and personal pain; hospital switching intent, budget, purchasing authority and current software; Mardaleishvili scope fit; Heart Centre's present fit; Vian approval route; direct professional contact permission; primary audit details.
Unmet: 20 qualifying posts, a verified top ten and five all-PASS buyers. A shorter evidenced result is preferable to an invented list.

These are potential customers based on public signals. Check every link yourself before you reply. More precisely, this result contains a provisional research queue. AI drafts; you decide whom to contact and send.

Sources and next checks

Opened 12 October 2026 | Clickable references, including exclusions

Exercise prompt document
S1 | C/Can: Quality cancer management (including contact affiliations)
S2 | C/Can: Plan Cancer Registry II
S3 | C/Can: Standardising cancer registry data collection
S4 | C/Can: Multidisciplinary management of breast and cervical cancers
S5 | BMG: Consilium Medulla director on DRG restrictions, 14 Jan 2026
S6 | BMG: Heart Centre director on regulatory/documentation concerns, 14 Jun 2023
S7 | BMG: reporting digital-health audit findings, 14 Feb 2025
S8 | Mardaleishvili: official services / specialised hospital scope
S9 | Todua: official services and patient app
S10 | American Hospital Tbilisi: official hospital website
S11 | Vian: official CMC profile and group affiliation
S12 | Mardaleishvili: official management team
S13 | Todua: official management team

Manual follow-up: original audit

State Audit Office original report page (did not load through web tool)

Open the audit from sao.ge if the direct page fails. Search for ციფრული ჯანდაცვის ეფექტიანობის აუდიტი. Copy the report date, the EMR/EHR compatibility paragraph and the hospital-reporting example; do not copy patient-identifying data. Paste with: Run Steps 1 to 4 again with this. This checks context, not buyer intent.

Other limits: Doctra live content was unreliable; do not use cached named testimonials as proof. jobs.ge id 748599 no longer showed a usable original posting on re-check. A Google Docs web fetch failed, but its content was successfully read using the authorised browser export. LinkedIn private groups and login-only profiles were not searched; no invented blocker or alleged warm path is reported.

Internal basis: marketing/mentor-explainers/justyna-product-explainer.json (8 October founder scope); marketing/research/2026-09-22-hospital-buyer-pain-points.md; founder instructions in this chat. Older AI-inclusive workshop copy was not treated as the current Phase 1 offer.